

إسم المريض رابعياً : فتحه عيد اسماعيل موسى

المحافظة : الجيزة الحي / المدينة : ابوالنمرس

رقم البطاقة : 25411152100625 رقم التليفون : 01010144587 تاريخ تسجيل الإستمارة : 24-09-2026

العمر(تاريخ الميلاد) : 15-11-1954 المهنة : بدون عمل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : اورام ليففاويه

نتائج الأبحاث : (مرفق)

التشخيص النهائي : اورام ليفاويه

خطة العلاج : أورام كيمائي وعلاج تدعيمى وتلطيفى [6396] قسم العلاج : الاورام

التعليق على التشخيص : -----

GR1-E
جورستان
روستای پلانی
لندی حسن
موسین رمضان
صحة البیابان المرفقة
للجنة و مدین

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطـة العـلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نفقات :

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

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Name: فتحه عيد اسماعيل موسى National ID: 25411152100625	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

A patient of lymphoma for Supportive and palliative care with follow-up

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

فتحية

عبد اسماعيل موسى قشمر

قرية نزلة الا شطر

مركز ابو النمر من - الجيزة



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101843194

١٢٣٠



٢٠٢٠/٠٩ ٢٥٤١١١٥٢١٠٠٦٢٥

ربة منزل

مسلمة

أنش



البطاقة سارية حتى ٢٠٢٧/٠٩/١٨



١٢٣٠

Name : fathia eid ismail mousa

Date : 11-03-2026 10:45

Branch : Portable

Medical No. : GRI-43935

Ref.Physician Prof : NCAP

Ins.Company :

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

No available previous images for comparison.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- Multiple bilateral axillary lymph nodes largest measuring 17 x 12 mm at the left axillary region.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Multiple para-aortic, aorto-caval, porta hepatis, mesenteric, porto-caval, bilateral common, internal and external iliac lymph nodes largest measuring 28 x 19 mm at the left external iliac region.
- Cirrhotic liver showing wavy outline and prominent left and caudate lobes. No hepatic focal lesion or dilated biliary radicles.
- Enlarged spleen showing no focal lesions.
- Suspected dilated porto-systemic collaterals at the splenic bed.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Normal plain C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Midline anterior abdominal wall subcutaneous lesion measuring 36 x 26 mm with calcific focus seen. No intra abdominal extension.
- No ascites.

Name : fathia eid ismail mousa

Branch : Portable

Ref.Physician Prof : NCAP

Date : 11-03-2026 10:45

Medical No. : GRI-43935

Ins.Company :

Opinion:

- ❖ Pelvi-abdominal lymphadenopathy as described.
- ❖ Cirrhotic liver with splenomegaly.
- ❖ Suspected portal hypertension for contrast enhanced study.
- ❖ Midline anterior abdominal wall subcutaneous lesion for histopathology.
- ❖ PET-CT is advised.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Name : fathia eid ismail mousa

Date : 13-03-2026 10:21

Branch : Portable

Medical No. : GRI-43935

Ref.Physician Prof : Neck

Ins.Company :

MULTISLICE NON CONTRAST C.T. EXAMINATION OF THE NECK REVEALED:

- Normal plain CT appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal plain CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Multiple subscentemetric bilateral deep cervical and submandibular LNs are noted.
- Multiple bilateral supraclavicular enlarged lymph nodes much more evident on the left side largest measuring 17 x 14 mm.

Opinion:

- **Multiple bilateral supraclavicular
lymphadenopathy for PET-CT assessment.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Patient Name: Fat'heya Eid Ismail Mousa
Exam : ultrasound ABDOMEN& PELVIS
Medical ID: 4025/2021

Date : 24/06/2025
Age : 70 Year(s)
Pat. ID: 43935

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: -

- Enlarged in size (about 17 cm at the right MCL), irregular outline, heterogeneous echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- Average size, normal wall thickness, no stones.
- CBD is not dilated.

Spleen: Enlarged in size (About 16 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site, normal echogenicity, no stone or backpressure changes, no solid or cystic lesion.
- Both ureters are noted dilated.

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: -:

- anti-verted, average sized uterus. Endometrial thickness measures about: mm, no focal lesions.

No adnexal lesions detected.

- No free fluid in the Douglas pouch.
- Para-aortic and pancreatic area: concealed by gases.

A subcutaneous cystic lesion is seen at midline anterior abdominal wall measures 3.5x1.5 cm with clear content and no soft tissue component.

Opinion:

- Hepatosplenomegaly with cirrhotic liver.
- Anterior abdominal wall subcutaneous simple cyst, for clinical correlation.

Best Regards,
Dr. Rasha Elbrahim

Patient Name: Fal'heya Eid Ismail Mousa	Date : 12/11/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 69 Year(s)
Medical ID: 4020/2023	Pat. ID: 43935

POST CONTRAST MULTI-SLICE CT SCAN OF THE NECK, CHEST, ABDOMEN, AND PELVIS: -

I Neck

- Left upper and lower cervical, posterior triangle and bilateral supraclavicular enlarged lymph nodes largest seen at left lower cervical level IV measuring 2.1 x 1.5 cm.
- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilaginous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- Heterogenous texture of thyroid gland for US correlation
- Normal appearance of the neck muscles & vessels.

IX Chest:-

- Bilateral enlarged axillary lymph nodes largest at right measuring 2.2 x 1.7 cm.
- Bilateral basal atelectatic bands.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

III Abdomen & pelvis:

- Multiple para aortic, aortocaval, portocaval, 7 retrocaval, porta hepatis and bilateral iliac enlarged lymph nodes largest at right external iliac measuring 2.8 x 1.9 cm.
- Enlarged spleen with no focal lesions seen.



Patient Name: Fat'heya Eid Ismail Mousa	Date : 12/11/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 69 Year(s)
Medical ID: 4020/2023	Pat. ID: 43935

- The liver is cirrhotic with wavy outline and enlarged left lobe. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The visualized part of the skeleton shows:
 - Spondylodegenerative changes.
 - Osteopenic texture of the examined bones.

Opinion:

- Multiple supra and infradiaphragmatic lymph nodes suspicious of lymphoma for histopathological correlation.

Dr. Ali Saber, MSc.

Signature Prof. Menna Talla Hanafy

21/11/2023 01:25 م

Patient Name : فتحيه عبد اسماعيل موسى
Patient No : 151139374
Sex : Female Age : 69 Y
Financial Cate : معهد ناصر
Department : Out Patient
Referring Doc :

Lab No. : 1271003
Request Date : 24-10-2023 12:09
Verification Date : 26-10-2023 12:14
Sample Type : Sample
Test Status : Routine
Analyzers :

Immunophenotyping Report

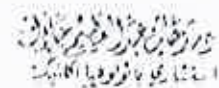
			Other Markers	Percentage	Remark
CD 11B			CD 34		
CD 11C	36.00	Positive	HLA-DR		
CD 117			CD 10	0.10	Negative
CD 64			TDT		
CD 61			CD 38	1.60	Negative
Glycophorin			CD 45		
Myeloperoxidase			CD 25	11.60	Negative
Cytoplasmic CD 13			CD 103	0.20	Negative
Cytoplasmic CD 33			Cyto.CD33		
			CD 81		
			CD 27		
			CD 41		

Comment: The gated CD19 cells in this PB sample constitute about~ 40.3% using CD19/SSC gating strategy.

It expresses an immunophenotype suggestive of ((B- CLL).

Haematology Consultant:
Prof. Dr/ Hanaa Afifi

Signature



Dr.Rehab Abdelazim Mohamed

Printing Date : 26/10/2023 12:23:19

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Printing By : Gehan refaat amin

Patient Name : فتحية عبد اسماعيل موسى
 Patient No : 151139374
 Sex : Female Age : 69 Y
 Financial Cate : معهد ناصر
 Department : Out Patient
 Referring Doc :

Lab No. : 1271003
Request Date : 24-10-2023 12:09
Verification Date : 26-10-2023 12:14
Sample Type : Sample
Test Status : Routine
Analyzers :

Immunophenotyping Report

B-Cell	Percentage	Remark	T-CELL	Percentage	Remark
CD19	100.00	Positive	CD 2		
CD20	83.30	Positive	CD 3		
CD22	25.20	Positive	CD 5	96.60	Positive
CD23	98.80	Positive	CD 7	0.10	Negative
CD79B			CD 56		
CD79a			TCR-ab		
S.IGM			TCR-dg		
CYT.IGM			CD 4		
K/CD 19	2.30	Negative	CD 8		
L/CD 19	0.10	Negative	CYT.CD 3		
CYT.CD 22			CD 5/CD 19 (coexp)	96.50	Positive
FMCT	0.20	Negative	CD 4/CD 8 (coexp)		
CYT0.CD13		Negative	CD 1n		
CYT.L					
CD 123	2.90	Negative			
CD 58					
NG 2					
CD 200	93.70	Positive			
CD 22/ CD 11c Co-exp					
K / CD 19 (Cyto)					
L / CD 19 (Cyto)					
Myloid Markers	Percentage	Remark			
CD 33					
CD 13					
CD14					
CD 15					

Signature

وَمِنْ مَنَافِعِهِ أَنْ يَسْتَفِيدَ مِنْهُ الْقَارِئُ بِإِذْنِ اللَّهِ تَعَالَى
مِنْ فَضْلِ الْإِسْلَامِ وَفَضْلِ الْوَلَدِ الْفَارِغِ مِنَ الْوَلَدِ الْفَارِغِ

Dr. Rehab Abdelazim Mohamed

Printing Date : 26/10/2023 12:23:19

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